

DYNA CLOXACILLIN DS 125

MAL19962517AZ

DESCRIPTION:

White coloured powder (orange suspension on adding water) with strawberry flavour.

EACH 5 ML OF THE RECONSTITUTED SUSPENSION CONTAINS:

Cloxacillin Sodium 136.67 mg equivalent to Cloxacillin 125 mg

PHARMACODYNAMICS:

Cloxacillin is a bactericidal antibiotic which inhibits bacterial cell wall synthesis, probably by acylation of membrane-bound transpeptidase enzymes. This prevents cross-linkage of peptidoglycan chains which is necessary for bacterial cell wall strength and rigidity. Also, cell division and growth are inhibited and lysis of susceptible bacteria frequently occurs. Rapidly dividing bacteria are most susceptible to penicillin group of antibiotics. Cloxacillin is resistant to staphylococcal penicillinase. It has an antibacterial spectrum similar to that of methicillin.

PHARMACOKINETICS:

Cloxacillin is incompletely absorbed from the gastrointestinal tract, and absorption is reduced by the presence of food in the stomach. After an oral dose of 500 mg, a peak plasma concentration of 7 to 15 mcg/ml is attained in fasting subjects in 1 to 2 hours. Absorption is more complete when given by intramuscular injection and peak plasma concentrations of about 15 mcg/ml have occurred 30 minutes after a dose of 500 mg. Doubling the dose can double the plasma concentration. About 94% of Cloxacillin in the circulation is bound to plasma proteins. Cloxacillin has been reported to have a plasma half-life of 0.5 to 1 hour. The half-life is prolonged in neonates. Cloxacillin crosses the placenta and is distributed into breast milk. There is little diffusion into the CSF except when the meninges are inflamed. Therapeutic concentrations can be achieved in pleural and synovial fluids and in bone.

Cloxacillin is metabolised to a limited extent, and the unchanged drug and metabolites are excreted in the urine by glomerular filtration and renal tubular secretion. About 35% of an oral dose is excreted in the urine and up to 10% in the bile. Cloxacillin is not removed by haemodialysis.

Plasma concentrations are enhanced by Probenecid. Reduced concentrations in patients with cystic fibrosis have been attributed to both enhanced tubular secretion and enhanced nonrenal clearance of Cloxacillin.

INDICATIONS:

Infections caused by staphylococci resistant to benzylpenicillin, including infections of the skin and soft tissues, bones, and joints, respiratory tract and urinary tract; otitis media, endocarditis, septicaemia, and meningitis. It is also used for mixed streptococcal and staphylococcal infections when the staphylococci are penicillin-resistant. It is also used for the prophylaxis of staphylococcal infections during major surgical procedures, particularly in cardiothoracic and orthopaedic surgery.

RECOMMENDED DOSE:

Adults: 250-500 mg every 6 hours daily.

Infants & Children up to 20 kg of body weight: Oral, 6.25-12.5 mg (base) per kg of body weight every 6 hours.

Children 20 kg of body weight & over: Usual adult dose.

- Note:**
- * Cloxacillin should not be used orally for initial treatment of severe life-threatening infections, but may be used as a follow-up therapy after parenteral penicillinase-resistant penicillin therapy.
 - * Patient should continue medicine for full course of treatment.
 - * Take medicine on empty stomach.
 - * Reduce dosage is required in patients with impaired renal function.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATION:

Patient hypersensitive to Cloxacillin or a history of allergic to penicillin group of drugs.

WARNING AND PRECAUTIONS:

Patients hypersensitive to other penicillins or cephalosporins or penicillamine may be hypersensitive to Cloxacillin. Patients known to be hypersensitive to penicillin should be given an antibiotic of another class. It should be given with caution to patients with a history of allergy especially to other drugs. Care is necessary if very high doses are given, especially if renal function is poor, because of the risk of neurotoxicity. Care is also necessary if large doses of the Potassium or Sodium salts are given to patients with impaired renal function or congestive heart failure. Renal & Haematological Systems should be monitored during prolonged and high-dose therapy. Contact with penicillin should be avoided since skin sensitisation may occur. If a skin rash occurs,

treatment should be discontinued. One of the dangers of penicillin therapy is the emergency of supra-infection by resistant species such as *Pseudomonas* or *Candida* which do not respond to penicillin therapy. The use of Cloxacillin by nursing mothers may lead to sensitization, diarrhoea, candidiasis, and skin rash in the infant. Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients on penicillin therapy with beta-lactams. Before initiating therapy with Dyna Cloxacillin DS 125, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Dyna Cloxacillin DS 125 must be discontinued immediately and appropriate alternative therapy instituted.

DRUG INTERACTIONS:

Cloxacillin Sodium has been reported to be incompatible with aminoglycosides, tetracyclines and other antimicrobial agents including erythromycin and polymyxin B sulphate.

PREGNANCY AND LACTATION:

There appeared to be no significant risk to the foetus from Cloxacillin when administered during pregnancy, although there was the possibility of sensitising the foetus during the second and third trimesters.

SIDE EFFECTS:

Side effects are uncommon and mainly of a mild and transitory nature.

Gastrointestinal reactions (e.g. diarrhoea, nausea or vomiting). Oral candidiasis (e.g. sore mouth or tongue). Pseudomembranous colitis. Allergic reactions (e.g. skin rashes; anaphylactic shock with collapse & sometimes death occurring within minutes; angioedema or bronchospasm; urticaria, fever, joint pains & eosinophilia). Intestinal nephritis (e.g. fever, rash, possibly decreased urine output, proteinuria, hematuria). Haematologic reactions (e.g. neutropenia - sore throat & fever).

SYMPTOMS AND TREATMENT OF OVERDOSE:

Problems of overdosage are unlikely to occur, if encountered they may be treated symptomatically.

Recommended treatment of hypersensitivity reactions consists of the following: Antihistamines; Systemic adrenocorticoids (if necessary). Serious anaphylactoid reactions require immediate emergency treatment, which consists of the following: Parenteral epinephrine; Oxygen; Intravenous adrenocorticoids and Airway management (including intubation).

Since there is no specific antidote, treatment of penicillin overdose should be symptomatic & supportive. Hemodialysis may aid in the removal of penicillins from the blood.

DIRECTION FOR MIXING:

Shake to loosen powder.

Add approximately half the final volume (30 ml or 50 ml) of freshly boiled and cooled water and **shake gently**.

Add more water until the final volume (60 ml or 100 ml) mark is reached and **shake well**.

Keep **refrigerated** (2-8°C) after reconstitution. To be used within **7 days** of mixing.

Complete the prescribed course.

PACKING/PACK SIZE(S):

Plastic bottles of 60 ml and 100 ml.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

SHAKE WELL BEFORE USE

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. 198001011897 (65683-V)

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